

CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: NancyPeriod: June 22 to July 22, 2024Report Date: July 23, 2024Coverage: 417/738h (56%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

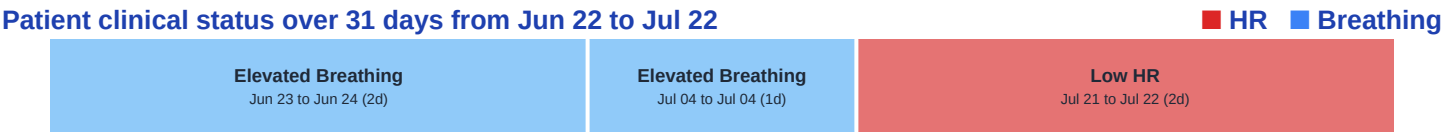
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: GREEN

Vital	Avg	Min	Max
Heart Rate	50 bpm	40 bpm	82 bpm
Breathing	17 brpm	9 brpm	27 brpm

Last 24 hours: four low heart rate episodes in the evening.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jul 21	10 PM to 11 PM	4h	4	Elevated Breathing	17	9	27	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jul 21	10 PM to 11 PM	4h	4	Low Heart Rate (brief)	44	40	49	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

10 episodic events Detected, spanning 10h total. Conditions: Elevated Breathing, Low Heart Rate. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
10	10h	Low Heart Rate	<1	56%

Major Findings: Stable through mid period; end of monitoring Low Heart Rate clustering (Jul 21 to Jul 22, 1.0/day).

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jul 21	3 AM to 4 AM	7h	7	Low Heart Rate	44	40	49	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Jun 23	3 PM to 4 PM	2h	2	Elevated Breathing	25	19	28	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jul 04	10 PM to 11 PM	1h	1	Elevated Breathing	25	25	25	Check SpO2 and lung sounds; assess for fluid overload or early infection

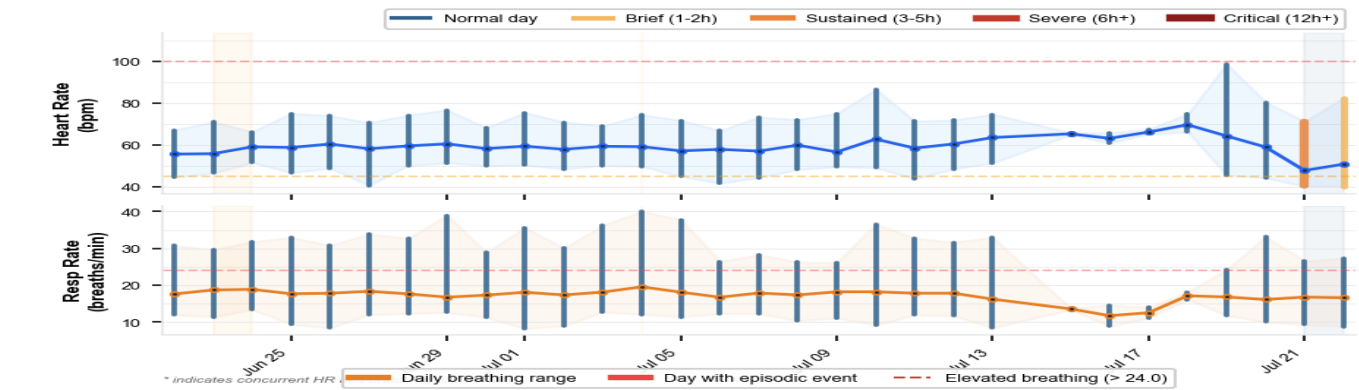
The P5 to P95 heart rate spread was 21 bpm (48 to 69), indicating significant cardiac variability.

Suggested Clinical Review Actions

- Low Heart Rate (avg 44 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Elevated Breathing (avg 25 brpm, peak 28 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Overall trajectory shows 3 unstable phases with 21 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

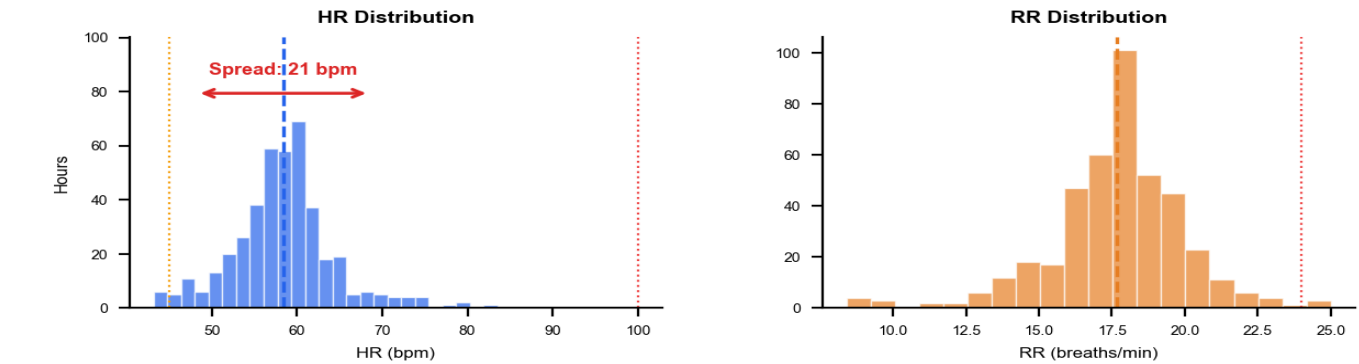
Clinical Pattern Observations:

- 1. High variability: HR spread 20 bpm (47 to 68).
- 2. Clustered pattern: Episodic events on 2 of 31 days (6%).
- 3. Nocturnal pattern: 4 of 6 eligible episodes during evening or night hours.

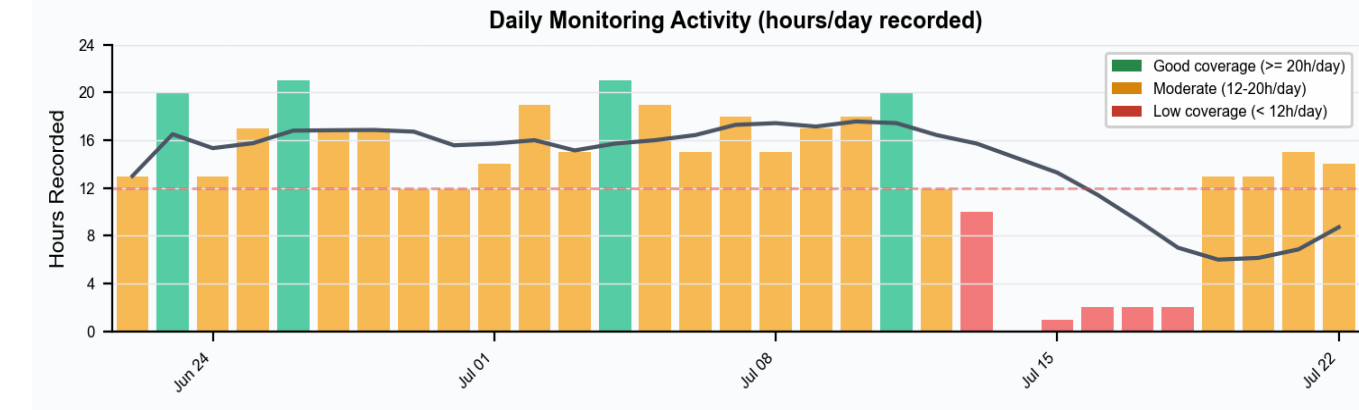


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.